

METROPOLITAN UNIVERSITY MEDICAL CENTER

Integrated Electronic Health Record | Clinical Dossier Summary

CONFIDENTIAL MEDICAL RECORD | RAG VALIDATION DATASET

SECTION 1: PATIENT DEMOGRAPHICS & ADMINISTRATIVE DATA

Patient Full Name: McAllister, Alistair Keith
Medical Record Number: MRN-44910-Z
Date of Birth: 03/14/1948
Age / Gender: 78 Years / Male
Marital Status: Married
Residential Address: 8842 Shady Pines Drive, Apt 4B, Metropolis, NY 10023
Attending Physician: Dr. Helena Vance, MD (Cardiology Group)
Admitting Service: Advanced Heart Failure / Cardiac Care Unit (CCU)
Admitting Insurance: Vanguard Senior Choice Plan
Policy Number: VNG-88310-M
Group ID: GRP-99214-X
Emergency Contact: Margaret McAllister (Spouse)
Contact Phone: (555) 014-7921
Admission Date: 05-18-2026
Discharge Date: 05/23/2026

SECTION 2: ACTIVE CLINICAL PROBLEM LIST

The following structural registry tracks the active, chronic, and acute primary diagnostic codes verified for this patient profile across clinical networks.

ICD-10 Code	Condition Description	Onset Date	Status
I50.32	Chronic Diastolic Congestive Heart Failure (CHF)	11/14/2021	Acute Decompensation
I48.91	Unspecified Atrial Fibrillation (A-Fib), Chronic	08-22-2019	Active (RVR Variant)
I10	Essential (Primary) Hypertension	04/11/2010	Active (Uncontrolled)
N18.30	Chronic Kidney Disease, Stage 3 (Unspecified)	2022-09-05	Baseline Insufficiency
I69.30	Sequelae of Ischemic Stroke (Prior CVA, 2024)	06/18/2024	Residual Left Hemiparesis
E78.5	Hyperlipidemia, Unspecified	03/12/2016	Active

SECTION 3: MEDICAL HISTORY & CHRONIC DISEASE TRAJECTORY

Cardiovascular Pathology:

Longstanding history of heart failure with preserved ejection fraction (HFpEF), initially diagnosed in November 2021 following an episode of unexplained exertional dyspnea. Echocardiography at that time demonstrated an LVEF of 52% with severe diastolic dysfunction and left atrial enlargement. Chronic atrial fibrillation was documented in 2019, managed via rate control and systemic anticoagulation. Systemic hypertension has been present for over 15 years, requiring a multi-drug regimen with variable compliance.

Neurological & Cerebrovascular History:

The patient suffered a right middle cerebral artery (MCA) ischemic cerebrovascular accident (CVA) on June 18, 2024. He was treated with intravenous tissue plasminogen activator (tPA) at an outside facility. Residual deficits include mild left-sided upper extremity weakness and a pronator drift. Cognitive function remains intact. Secondary prevention is maintained via high-intensity statin therapy and anticoagulation.

Renal & Metabolic Background:

Progressive renal impairment has been tracked over the past four years, establishing a stable baseline serum creatinine between 1.35 mg/dL and 1.45 mg/dL, qualifying as Chronic Kidney Disease (CKD) Stage 3. Metabolic history is notable for

hyperlipidemia, controlled with Atorvastatin.

SECTION 4: CURRENT MEDICATION REGIMEN (ADMISSION TO DISCHARGE)

The following profile lists the long-term outpatient prescriptions reconciled at admission alongside acute inpatient modifications.

Medication Name	Dosage	Route	Frequency	Prescribing MD	Clinical Indication
Carvedilol	12.5 mg	PO	Twice Daily	Dr. H. Vance	CHF / A-Fib Rate Control
Apixaban	2.5 mg	PO	Twice Daily	Dr. H. Vance	Stroke Prophylaxis (A-Fib)
Lisinopril	10 mg	PO	Once Daily	Dr. H. Vance	Hypertension / CHF
Atorvastatin Sodium	40 mg	PO	Once Daily QHS	Dr. H. Vance	Secondary CVA Prevention
Amlodipine Besylate	5 mg	PO	Once Daily	Dr. H. Vance	Calcium Channel Blocker
Furosemide	40 mg	IV	Every 12 Hours	Dr. H. Vance	Acute Diuresis (Volume)
Lasix	40 mg	IV	Twice Daily	Resident Floor	Fluid Retention Management
Potassium Chloride	20 mEq	PO	Once Daily	Dr. H. Vance	Electrolyte Repletion
Allopurinol	100 mg	PO	Once Daily	Dr. E. Thorne	Gout Prophylaxis (Home)
Pantoprazole Sodium	40 mg	PO	Once Daily AC	Dr. H. Vance	Gastroprotection
Cyanocobalamin	1000 mcg	PO	Once Daily	Dr. E. Thorne	Dietary Supplementation

Clinical Pharmacy Note: Furosemide and Lasix were documented concurrently within the computerized physician order entry system during the transition from the emergency department to the cardiac floor, presenting a duplicate therapeutic entry stream.

SECTION 5: ALLERGIES & ADVERSE DRUG REACTIONS

Allergen / Agent	Reaction Type	Clinical Manifestation / Severity
Sulfa (Sulfonamides)	True Drug Allergy	Diffuse maculopapular rash, angioedema (Severe)
Contrast Media (Iodine)	Pseudoallergic	Generalized pruritus, mild urticaria, flushing

SECTION 6: CARDIOLOGY CONSULTATION NOTE

Date of Encounter: May 18, 2026

Consulting Cardiologist: Dr. Lawrence Vance, MD, FACC

Requesting Service: Emergency Medicine

Reason for Consultation: Acute hypertensive heart failure decompensation triggered by atrial fibrillation with rapid ventricular response (RVR).

History of Present Illness:

The patient is a 78-year-old male with a known history of chronic diastolic heart failure (HFpEF), chronic atrial fibrillation, hypertension, Stage 3 CKD, and a prior stroke who presents with a 4-day history of progressive dyspnea on exertion, 3-pillow orthopnea, paroxysmal nocturnal dyspnea, and gross bilateral lower extremity swelling. He notes an acute 12-pound weight gain over the past week. He denies chest pain, jaw discomfort, or new focal neurological deficits.

Physical Examination Findings:

Vitals: BP 164/98 mmHg, HR 118 bpm (irregularly irregular), RR 24 breaths/min, Temp 98.4 F, SpO2 88% on room air, improving to 93% via 3L nasal cannula.

General: Elderly male in moderate respiratory distress, speaking in broken sentences while leaning forward.

Heent: Jugular Venous Distension (JVD) noted up to 13 cm at a 45-degree angle. Hepatojugular reflux is positive.

Cardiovascular: Tachycardic, irregularly irregular rhythm. Distinct S3 gallop appreciated at the apex. 2/6 holosystolic murmur at the lower left sternal border. Peripheries are cool with delayed capillary refill.

Pulmonary: Prominent rales and coarse crackles extending bilaterally to the mid-lung fields. Accessory muscle use noted.
Extremities: 3+ pitting edema present symmetrically up to the level of the mid-thigh.

Diagnostic Synthesis & Clinical Impression:

This is a classic presentation of acute on chronic volume-overload heart failure (diastolic phenotype) precipitated by a loss of ventricular filling time due to rapid atrial fibrillation. The patient's systemic afterload is highly elevated (systolic BP >160 mmHg), driving pulmonary venous congestion. High-sensitivity Troponins are flat, effectively ruling out an acute Type 1 transmural myocardial infarction. The patient's stroke risk profile is elevated (CHA2DS2-VASc score of 5), requiring continuous anticoagulation, which is appropriately maintained on a renal-adjusted dose of Apixaban due to chronic kidney disease stage 3.

Recommendations:

1. Transfer to the Cardiac Care Unit (CCU) for continuous telemetry monitoring.
2. Initiate aggressive intravenous loop diuresis with Furosemide 40 mg IV BID. Target a net negative fluid balance of 1.5 to 2.0 Liters every 24 hours.
3. For rate control, continue Carvedilol 12.5 mg PO BID; avoid acute intravenous beta-blocker boluses given the current decompensated state.
4. Serial basic metabolic panels every 12 hours to monitor for pre-renal azotemia and electrolyte depletion.

SECTION 7: LONGITUDINAL PHYSICIAN PROGRESS NOTES

Encounter Date: 05-19-2026

Provider: Dr. Helena Vance, MD (Cardiology)

Subjective: Patient reports a slight improvement in breathing comfort after initial IV diuretic doses. He was able to sleep with the head of the bed elevated at 30 degrees without waking up gasping. No chest pain.

Objective: Vitals: BP 148/88 mmHg, HR 98 bpm (irregular), RR 20/min, SpO2 95% on 2L nasal cannula. Net fluid output over the last 18 hours is negative 2.1 Liters.

Assessment & Plan: Acute on chronic diastolic heart failure. Showing appropriate initial response to diuresis. Continue Furosemide 40 mg IV BID. Potassium level has trended down to 3.6 mEq/L; initiate oral Potassium Chloride 20 mEq daily repletion. Monitor renal function closely as intravascular volume contracts.

Encounter Date: 2026-05-20

Provider: Dr. Helena Vance, MD (Cardiology)

Subjective: Mr. McAllister reports significant relief. He states his breathing is "80% back to normal" and notes that his legs feel lighter. He has been ambulatory to the bedside commode without distress.

Objective: Vitals: BP 126/74 mmHg, HR 74 bpm (irregularly irregular), RR 16/min, SpO2 96% on room air.

Physical Exam: Lungs reveal significantly decreased crackles, now restricted to the lung bases only. JVD down to 6 cm. Minimal 1+ trace pitting edema over the ankles.

Assessment & Plan: Diastolic heart failure volume overload is resolving rapidly. Telemetry shows stable, controlled atrial fibrillation with a ventricular rate averaging in the low 70s. However, current basic metabolic panel reveals a significant rise in serum creatinine to 1.85 mg/dL from an admission baseline of 1.42 mg/dL. This is indicative of Type 2 Cardiorenal Syndrome / Acute Kidney Injury (AKI) stage 1 secondary to aggressive diuresis. Will down-titrate Furosemide to 20 mg IV daily to preserve glomerular filtration pressure.

Encounter Date: 05/21/2026

Provider: Dr. Robert Mercer, MD (Internal Medicine / Floor Resident)

Subjective: Patient is resting comfortably in bed. No respiratory complaints or shortness of breath noted during assessment.

Objective: Vitals: BP 154/92 mmHg, HR 76 bpm, RR 18/min, Temp 98.6 F.

Physical Exam: Patient appears in mild respiratory distress while resting, utilizing accessory muscles to breathe. Lungs are completely clear to auscultation bilaterally with normal respiratory excursion.

Plan: Renal clearance appears stable. Defer all medication alterations at this time. Maintain present floor plan.

Clinical Reasoning Discrepancy: The resident note written by Dr. Mercer on 05/21/2026 contains an internal contradiction, stating the patient is "resting comfortably" with "no respiratory complaints" in the subjective portion, while simultaneously documenting "appears in mild respiratory distress" with "accessory muscle use" in the physical examination section.

SECTION 8: NURSE NOTE

Date: May 22, 2026

Shift: Day Shift (07:00 - 19:00)
Log Author: S. Jenkins, RN
Patient received resting comfortably in bed on room air. Awake, alert, and oriented to person, place, time, and situation. Left-sided residual weakness noted from prior stroke, but patient is able to reposition himself in bed with minimal assistance. Left forearm peripheral IV line is patent, clean, and intact with no signs of infiltration or erythema.

The morning oral medication pass was completed successfully. Patient tolerated all oral intake well. Strict intake and output (I/O) tracking is maintained. Total fluid intake for the shift was 1,100 mL (oral fluids and meals); total urine output was 2,450 mL via urinal, representing a continued net negative volume status. Daily morning weight was recorded at 184 lbs, showing a down-trend from 198 lbs at admission.

Cardiovascular assessment demonstrates a flat jugular vein, soft abdomen, and warm distal extremities. Distal pulses (radial and dorsalis pedis) are palpable at 2+ bilaterally. No new complaints of chest pain, palpitations, or dizziness. Patient was educated on turning slowly when rising from the bed to prevent orthostatic lightheadedness. Skin is intact across all bony prominences; barrier cream applied to sacral region. Will continue to monitor telemetry and renal labs.

SECTION 9: ELECTROCARDIOGRAM (ECG) INTERPRETATION

ECG Tracking ID: ECG-2026-99124
Acquisition Date: 05-18-2026 (08:14 AM)
Ventricular Rate: 112 bpm
Atrial Rate: Undetermined (Fibrillatory waves)
PR Interval: N/A
QRS Duration: 104 ms
QTc Interval: 458 ms
Axis: Left Axis Deviation (-35 degrees)

Interpretation:

Atrial Fibrillation with Rapid Ventricular Response (RVR). Tachycardia is present. No premature ventricular complexes observed. QRS complexes display voltage criteria meeting the Cornell product threshold for Left Ventricular Hypertrophy (LVH), consistent with a chronic history of severe systemic hypertension. Prominent ST-segment depressions measuring 1.5 mm are noted in the lateral leads (V4, V5, V6), which are indeterminate and likely represent secondary repolarization abnormalities due to rate and hypertrophy, rather than acute transmural ischemia. No hyperkalemic peaked T-waves or acute ST-segment elevations are visualized. Comparison with a previous baseline ECG from January 2026 demonstrates a loss of sinus rhythm rate control.

SECTION 10: LABORATORY DIAGNOSTIC PROFILES

Specimen Facility: MUMC Central Pathology Group
Status: Final Verified Registry

Component Test	Admission (05-18)	Mid-Care (05-20)	Reference Range
B-Type Natriuretic (BNP)	1,450 pg/mL	680 pg/mL	< 100 pg/mL (CRITICAL HIGH)
Serum Creatinine	1.42 mg/dL	1.85 mg/dL	0.70 - 1.30 mg/dL (HIGH)
eGFR (CKD-EPI)	46 mL/min/1.73m²	33 mL/min/1.73m²	>= 60 mL/min/1.73m² (LOW)
Serum Sodium	136 mEq/L	138 mEq/L	136 - 145 mEq/L
Serum Potassium	3.9 mEq/L	3.6 mEq/L	3.5 - 5.1 mEq/L
Blood Urea Nitrogen (BUN)	28 mg/dL	49 mg/dL	7 - 20 mg/dL (ELEVATED)
Fast Serum Glucose	104 mg/dL	112 mg/dL	70 - 99 mg/dL (ELEVATED)
White Blood Cell (WBC)	7.2 x10 ³ / µL	6.9 x10 ³ / µL	4.5 - 11.0 x10 ³ / µL
Red Blood Cell (RBC)	4.10 x10 ⁶ / µL	4.15 x10 ⁶ / µL	4.30 - 5.90 x10 ⁶ / µL
Hemoglobin (Hgb)	12.9 g/dL	13.1 g/dL	13.5 - 17.5 g/dL
Hematocrit (Hct)	38.8 %	39.2 %	41.0 - 50.0 % (LOW)

Component Test	Admission (05-18)	Mid-Care (05-20)	Reference Range
Platelet Count	198 x10 ³ / μ L	202 x10 ³ / μ L	150 - 450 x10 ³ / μ L
LDL Cholesterol	84 mg/dL	Not Retested	< 100 mg/dL (Targeted)
HDL Cholesterol	41 mg/dL	Not Retested	> 40 mg/dL
Triglycerides	135 mg/dL	Not Retested	< 150 mg/dL

Ambiguous Reference Lab Note: An isolated laboratory text stream file labeled "CARD-BNP-CONC" logged an un-annotated follow-up metric value of "340" on 05-22-2026. The electronic filing failed to clarify if this represents a standard BNP parameter or an NT-proBNP index.

SECTION 11: IMAGING & RADIOLOGY REPORT

Document Type: Diagnostic Radiograph (Chest X-Ray, AP View)

Examination Date: March 18, 2026

Accession Number: RAD-2026-4412-A

Ordering Physician: Dr. Helena Vance, MD

Interpretation By: Dr. Sarah Vance, MD (Department of Diagnostic Radiology)

Clinical Indication:

Acute onset respiratory distress, hypoxia, and suspected volume overload in an elderly male with a history of diastolic heart failure.

Findings:

The chest radiograph was acquired at the bedside using a portable anteroposterior (AP) technique. Pulmonary expansion is limited due to patient positioning and body habitus.

The cardiac silhouette is diffusely enlarged, with the cardiothoracic ratio measuring approximately 58%, which is a prominent increase from previous film examinations. There is prominent engorgement of the upper lobe pulmonary vasculature, demonstrating significant cephalization. Diffuse, bilateral interstitial markings are prominent throughout both lung fields, with associated alveolar opacification in the dependent lower lobes, consistent with severe pulmonary edema.

Hilar structures are blurred and prominent. Kerley B lines are visible at the right lung periphery. Sizable, symmetric bilateral pleural effusions are present, obscuring the costophrenic angles bilaterally. The mediastinal contour appears stable and within normal limits. The trachea is midline. A temporary peripheral line is noted traversing the left upper extremity, terminating appropriately. No focal consolidation suggestive of an active lobar pneumonia is identified. Bony structures are osteopenic but intact; no acute fractures are visualized.

Diagnostic Impression:

1. Cardiomegaly with severe pulmonary venous congestion and diffuse alveolar/interstitial pulmonary edema.
2. Moderate bilateral pleural effusions, right greater than left.
3. No radiographic evidence of acute pneumothorax or localized bacterial consolidation.

SECTION 12: INPATIENT HOSPITAL DISCHARGE SUMMARY

Principal Discharge Diagnosis:

Acute on Chronic Diastolic Congestive Heart Failure (Resolved to baseline volume status)

Secondary Diagnostic Correlates:

Chronic Atrial Fibrillation with controlled ventricular response, Secondary Acute Kidney Injury (AKI Stage 1, resolving) on underlying Chronic Kidney Disease Stage 3, Essential Hypertension, History of Ischemic Cerebrovascular Accident (CVA) with left residual hemiparesis, Hyperlipidemia.

Hospital Course Summary:

Mr. McAllister presented to the Emergency Department in severe fluid overload and respiratory distress, driven by an acute exacerbation of his diastolic heart failure, which was precipitated by atrial fibrillation with rapid ventricular response (RVR). He was stabilized on supplemental oxygen and transferred to the Cardiac Care Unit (CCU) under the care of the cardiology service. Intensive intravenous loop diuresis was achieved via Furosemide 40 mg IV BID. Over a 5-day admission window, the

patient responded favorably, with a total documented net fluid reduction of approximately 14 pounds, resulting in the total clearance of his pulmonary edema, resolution of orthopnea, and stabilization of his oxygen saturation to room air.

On day 3 of admission, the patient's serum creatinine peaked at 1.85 mg/dL, representing a temporary pre-renal AKI secondary to rapid diuresis. The diuretic dose was lowered to 20 mg IV daily, which successfully halted the renal decline. At the time of discharge, his serum creatinine has down-trended to 1.48 mg/dL, approaching his chronic renal baseline. His ventricular rate has regularized to a stable range of 70 to 80 bpm using oral Carvedilol optimization. He remains neurologically stable at his baseline.

Discharge Medications & Outpatient Plan:

The patient is instructed to stop all intravenous medication orders and adhere to the following oral outpatient transition schedule:

1. Furosemide 40 mg Tablet: Take 1 tablet by mouth daily in the morning. (Transitioned from inpatient IV formulation. Strict compliance with morning weights is mandated).
2. Carvedilol 12.5 mg Tablet: Take 1 tablet by mouth twice daily (morning and evening). Do not skip doses.
3. Apixaban 2.5 mg Tablet: Take 1 tablet by mouth twice daily. Essential for stroke prophylaxis.
4. Lisinopril 10 mg Tablet: Take 1 tablet by mouth daily.
5. Atorvastatin 40 mg Tablet: Take 1 tablet by mouth daily at bedtime.
6. Potassium Chloride 20 mEq Tablet: Take 1 tablet by mouth daily with breakfast.

Mandated Follow-Up Protocols:

- * The patient must obtain a repeat basic metabolic panel (specifically checking Serum Creatinine and Electrolytes) at his primary care clinic within 48 to 72 hours of discharge to ensure renal stability.
- * A formal outpatient follow-up appointment is confirmed with Dr. Helena Vance at the Heart Institute on June 04, 2026.
- * The patient and his family were counseled on strict dietary restrictions: sodium must be kept under 2 grams daily, and total oral fluid intake must be capped at 1.5 Liters every 24 hours. The patient was instructed to contact the clinic immediately if he experiences a weight gain exceeding 3 pounds within a 24-hour window or an increase in shortness of breath.

***** END OF CLINICAL RECORD PACKET • ST. JUDE MEMORIAL HOSPITAL *****